Renal Outcomes with Glucoren plus Cardivex in Diabetic Hypertensive Patients

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Objective

To compare kidney function trajectories in patients with type 2 diabetes and hypertension treated with Cardivex plus Glucoren versus Cardivex alone, using a fictional multicenter observational registry.

Cohort

1,204 patients were followed for 24 months: 598 on combination therapy and 606 on Cardivex monotherapy. Mean baseline eGFR was 68 mL/min/1.73 m2 and median urine albumin-to-creatinine ratio (UACR) was 54 mg/g.

Outcomes

Mean annual eGFR decline was 1.9 mL/min/1.73 m2 with the combination versus 3.4 mL/min/1.73 m2 with Cardivex alone. UACR fell by 31% versus 12%. The composite renal endpoint (40% eGFR decline, dialysis or renal death) occurred in 4.2% versus 7.9% of patients (adjusted hazard ratio 0.54; 95% CI 0.34 to 0.86).

Safety

Hyperkalemia was less frequent with the combination (1.5% vs 2.6%). Volume depletion events were more frequent (2.2% vs 0.8%).

Limitations

Observational design; residual confounding cannot be excluded.

Summary table

24-month outcome	Glucoren + Cardivex	Cardivex alone
Annual eGFR decline	1.9	3.4
UACR change	-31%	-12%
Composite renal endpoint	4.2%	7.9%
Hyperkalemia	1.5%	2.6%